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Linh Tran

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EDUCATION

Johns Hopkins Bloomberg School of Public Health 2023–
PhD Candidate, Health Economics and Policy
Advisors: Matt Eisenberg and Karen Shen
Dissertation: Large Online Mental Health Practices and Demand and Supply of Mental Health Services
Committee Members: Matt Eisenberg, Karen Shen, Sachini Bandara, Rheanna Platt

Princeton University 2020
BA, Economics, *summa cum laude*

RESEARCH INTERESTS

Fields: applied microeconomics, health economics

Interests: mental health services, provider behavior, Medicaid

JOB MARKET PAPER

"Digital Platforms and Provider Behavior: Evidence from Mental Health Services"

The U.S. is facing a large and persistent treatment gap in mental health services. Administrative frictions exacerbate this dynamic. For providers, administrative frictions, such as onerous credentialing process to join insurance networks and complex administrative tasks, contribute to burnout and reluctance to accept health insurance. For patients, persistent inaccuracies in provider directories prevent them from getting insurance-covered mental health services. This project examines how alleviating administrative frictions impacts the market for mental health services by studying the rise of large online mental health practices (hereafter "LOMPs"), platforms that organize providers under a single group practice and contract with payers on their behalf, and provide a convenient platform for patients to search and match with in-network providers. Using a state's all payer claims database, I examine the effects of LOMPs on providers' practice patterns, such as patient panels and reimbursements, and on patients' access to mental health services and quality of care.

PUBLICATIONS

Coleman Drake, Linh Tran, and Matthew Eisenberg. Medical Cannabis Certifications After Pennsylvania Added Anxiety Disorders as a Qualifying Condition. *Annals of Internal Medicine*, 178(8):1209–1211, August 2025.

We study the impact of adding anxiety disorders as a qualifying condition for medical cannabis certifications in Pennsylvania on the distribution of qualifying conditions. We find that the share and number of medical cannabis certifications with anxiety disorders as a qualifying condition rapidly increased upon this policy change.

UNDER REVIEW

"A Substantial Share of Medicare Patients Likely Pay Cash for Psychiatric Care: Findings from A Novel Algorithm Using Claims" (first-author, with Karen Shen and Mark Meiselbach, revise and resubmit at Health Services Research)

Low insurance acceptance rates among psychiatrists may require patients to pay fully out-of-pocket ("cash pay") for care, potentially imposing financial burden and deterring treatment. Yet, research quantifying the prevalence of cash pay among psychiatry patients and describing the characteristics of those patients is limited. Using Medicare claims for a random 20% sample of traditional Medicare beneficiaries prescribed mental health medications by a psychiatrist or primary care physician (PCP) between 2016 and 2020, we develop a novel algorithm to identify patients likely paying cash for psychiatrist services and describe their demographic and clinical characteristics. 15.2% of Medicare beneficiaries who saw a psychiatrist likely paid cash for their visits, compared to 2.5% of PCP patients receiving similar medications. Patients who likely paid cash for psychiatrist services were more likely to reside in high-income areas and had prescriptions and diagnoses consistent with less serious mental illnesses. The prevalence of paying cash for psychiatrist services varied more than 7-fold across metropolitan areas. The findings underscore potential inequities in care allocation and highlight the need for ongoing policy efforts to improve insurance acceptance among psychiatrists.

WORKS IN PROGRESS

"Corporatization in Inpatient Psychiatry and Its Implications for Cost and Quality of Care" (with Karen Shen and Adam Sacarny)

We examine the impact of openings of large for-profit psychiatric hospitals on access and quality of care. Inpatient psychiatric care is particularly important for patients with serious mental illness. The rise of for-profit psychiatric hospitals may increase supply of specialized inpatient psychiatric care, thereby addressing unmet need. However, patients may be harmed if these new facilities are lower in quality. We leverage administrative claims data and a difference-in-differences approach to examine the effects of new psychiatric facilities and new for-profit psychiatric facilities on access to inpatient psychiatric care and patient outcomes.

"Characteristics of Patients Using Crisis Care Services in Arizona" (with Karen Shen and Matt Eisenberg)

As mental health crises become increasingly common and contribute to overcrowding of emergency departments (ED), states are expanding mental health crisis care as a community-based and cost-effective alternative to the ED. Understanding utilization patterns and outcomes for patients receiving care at crisis care centers versus the ED provides crucial evidence as policymakers consider funding these services. Using Medicaid claims from the 2021 Transformed Medicaid Statistical Information System Analytic Files, we explore how characteristics of people using crisis care differ from those using emergency department for mental health-relation reasons in Arizona. We identify a set of crisis centers and compare the distribution of patient diagnoses and

demographics among mental health-related or substance use-related episodes involving a crisis center, an emergency department, or both. Among mental health-related episodes, those involving crisis care are more likely to have diagnoses of schizophrenia and bipolar disorder than ED-only episodes. Among substance use-related episodes, opioid and stimulant disorders were more common among crisis episodes.

"Impact of Medicaid expansion on transport refusals after nonfatal overdoses: a national analysis of EMS data" (with Jirka Taylor, Alexander McCourt, and Karen Shen)

Transport refusals after nonfatal overdoses may increase risk of adverse events. Yet, concerns over costs of emergency care may deter patients from receiving appropriate care. Using 2018-2024 data from the National Emergency Medical Services Information, we examine whether increased insurance coverage through the Medicaid expansion affected rates of transport refusal rates for nonfatal overdoses involving emergency medical services ("EMS"). We use staggered difference-in-differences to compare trends in refusals between states that expanded Medicaid during the study period versus states that have yet to expand Medicaid. Preliminary evidence suggests that Medicaid expansion is associated with a reduction in transport refusal rates.

TEACHING EXPERIENCE

Johns Hopkins University

Graduate Teaching Assistant

Applied Microeconomics for Policymaking	Fall 2024
Health Economics	Fall 2024, Fall 2025
Introduction to Health Economics	Spring 2025
Econometric Methods for the Evaluation of Health Programs	Spring 2026

Princeton University

Peer Tutor	Spring 2017–Spring 2019
Undergraduate Course Assistant	Fall 2017–Spring 2020

CONFERENCE PRESENTATIONS

2026: "Estimating the Size and Characteristics of the Cash-Pay Market for Psychiatry in Claims Data", ASHEcon Annual Conference

SERVICE

Academic Policy and Admissions Committee 2023–2025

OTHER EMPLOYMENT

Quantitative Economic Solutions, LLC 2021–2023
Associate Consultant

Stanford Law School 2020–2021
Research Fellow to Dr. Alison Morantz

HONORS AND FELLOWSHIP

Princeton University

Summer Research Grant 2019

Phi Beta Kappa 2020

PERSONAL INFORMATION

Citizenship: Vietnamese